Chapter 1: The Six-Pillar Framework

The Six-Pillar Framework

Why Transformation Fails When Any Pillar Is Neglected, and How All Six Must Move Together

> *Chapter 1 establishes the six-pillar framework that organizes this entire book. It answers three questions: what each pillar is, how the pillars interact with each other, and why transformation fails when any pillar is neglected. It also introduces the Vermont thread that runs through every subsequent chapter — establishing Vermont's system in broad strokes before Chapters 2 and 3 develop the detail.*

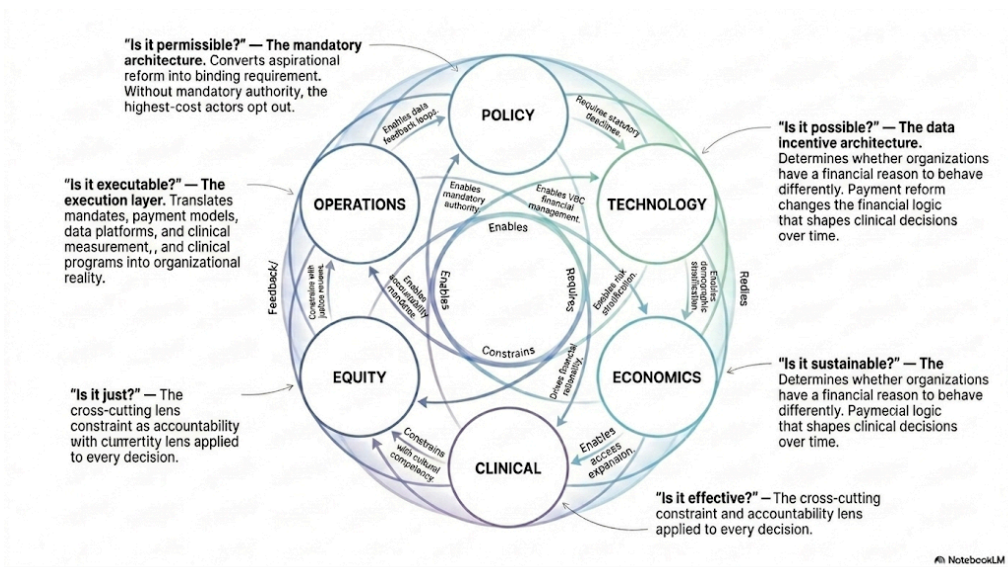

Figure 1.1 — The six pillars:

The Problem with Single-Pillar Thinking

In 2010, the Affordable Care Act created the Center for Medicare and Medicaid Innovation with a mandate to test new payment and delivery models and scale what worked. Over the following fifteen years, CMMI launched more than fifty models, investing billions of dollars and the attention of thousands of healthcare organizations. The results were modest. A 2023 evaluation found that the vast majority of CMMI models had not achieved statistically significant reductions in spending or improvements in quality. Most were discontinued. A handful showed promise and were expanded.

The pattern of failure across CMMI models is instructive. Models that addressed only the Economics pillar — changing what Medicare paid without addressing how care was delivered or whether providers had the infrastructure to respond — generated financial pressure without behavioral change. Models that addressed only the Clinical pillar — redesigning care coordination or care management programs — produced clinical improvements that the payment system then penalized by reducing volume-based revenue. Models that required Technology pillar investment — electronic health records, population health platforms, data exchange — found that providers lacked the operational capacity to use the tools effectively.

The lesson is not that payment reform, clinical redesign, or technology investment is wrong. It is that each intervention, pursued in isolation, runs into the constraints created by the pillars it ignores. This is the core analytical argument of the six-pillar framework: healthcare transformation is a system problem, and system problems require system solutions.

Every major healthcare transformation framework in the past thirty years has identified the same list of necessary conditions. Pay differently. Redesign care delivery. Build data infrastructure. Address social determinants. Ensure equity. Execute operationally. The list is right. The reasoning behind it is right. And yet the transformation that the list is supposed to produce has not arrived — not in most states, not at national scale, not with the speed and completeness the evidence has long indicated is possible.

The problem is not the list. The problem is treating it as a list.

A checklist is a set of independent items. You can check them in any order. Checking one does not affect the others. Failing to check one does not prevent you from checking the rest. A checklist of healthcare transformation requirements implies that you can make progress on any one of these independently, and that partial completion is partial progress. This is wrong. The six pillars of the six-pillar framework are not independent items on a checklist. They are a system of interdependent components in which each pillar's effectiveness depends on the others, in which the failure of any single pillar cascades through the system, and in which the order and timing of interventions matters as much as the content of those interventions. Treating the six pillars as a checklist is precisely how decades of technically correct healthcare reform recommendations have produced inadequate results.

> *"A policy intervention that passes question one but fails questions three, four, five, or six is not a solution — it is a well-intentioned mistake waiting to be made."*

— HTR Framework Documentation

The Six Pillars Defined

The six pillars are not equal in their causal priority — payment reform (Economics) and enabling legislation (Policy) are preconditions that the other pillars depend on — but they are equal in their necessity. A transformed healthcare system requires all six to be functional.

Policy	The legislative and regulatory environment that creates the mandatory architecture for transformation. Policy establishes what actors must do, what they are prohibited from doing, and what governance structures ensure accountability. Voluntary reform operates within the Policy pillar's constraints; structural transformation requires it to change. Vermont application: *Acts 167 and 68, the AHEAD Model State Agreement, and the Statewide Strategic Plan mandate are Vermont's Policy pillar interventions. Each converts voluntary reform into statutory obligation.*

Economics	The payment and financial architecture that determines what behaviors the system rewards. The Economics pillar encompasses payment reform (from fee-for-service to value-based and global payment), financial modeling, total cost of care management, and the hospital financial sustainability that transformation requires. Vermont application: *Reference-based pricing (FY2027), global hospital budgets (FY2028-2030), and the AHEAD Model's total cost of care accountability are Vermont's Economics pillar interventions.*
Operations	The organizational capacity, management infrastructure, administrative simplification, and implementation discipline that converts strategy into executed programs. The Operations pillar is where transformation plans become real or

	remain documents — the graveyard of well-designed reforms that could not be implemented. Vermont application: *The 14-hospital transformation planning process, RHRC technical assistance, the AHS HSA Coordinator model, the Division of Planning and Effectiveness, and EMS regionalization are Vermont's Operations pillar investments.*
Clinical	The care delivery models, quality measurement frameworks, clinical workforce design, and patient care programs that determine how care is actually delivered. The Clinical pillar is where payment reform becomes patient outcomes — or fails to. Vermont application: *The Blueprint for Health (all-payer PCMH program), Collaborative Care Model behavioral health integration, CCBHC expansion, and the primary care investment strategy are Vermont's Clinical pillar interventions.*
Equity	The programs, investments, and accountability structures that ensure transformation benefits reach all populations — and specifically those most harmed by the current system's failures. Equity is not a soft afterthought; it is an analytical requirement. A transformation that reduces average costs while widening disparities has failed. Vermont application: *Geographic equity across Vermont's 14 HSAs, SDOH investment through the EAST Fund and Blueprint HRSN screening, rural access preservation in the RBP and global budget design, and health equity metrics in Act 167/68 reporting are Vermont's Equity pillar interventions.*
Technology	The data infrastructure, health information exchange, analytical platforms, and clinical technologies that make population health management possible at scale. The Technology pillar is the information substrate on which all other pillar functions depend — you cannot manage what you cannot measure. Vermont application: *VHCURES (all-payer claims database), VITL/VHIE (health information exchange), the AHS-GMCB analytics vendor, AI scribe grants, and Vermont's Clinically Integrated Network are Technology pillar investments.*

The table below characterizes each pillar's diagnostic function and structural role in the system — not as a repetition of the definitions above, but as a specification of what each pillar produces when functioning and what question it answers in the transformation logic.

Pillar	Diagnostic question	Structural role in the system	What it produces when functioning
Policy	Is it permissible?	The mandatory architecture. Converts aspirational reform into binding requirements. Without mandatory authority, the highest-cost actors opt out and voluntary reform achieves marginal results.	Statutory mandates that eliminate voluntary opt-out; accountability timelines that cannot be deferred; federal-state agreements that commit both parties to specific outcomes.

Pillar	Diagnostic question	Structural role in the system	What it produces when functioning
Economics	Is it sustainable?	The incentive architecture. Determines whether organizations have a financial reason to behave differently. Payment reform does not change clinical behavior directly — it changes the financial logic that shapes clinical decisions over time.	Payment models in which population health management is financially rational; price structures that break the commercial cross-subsidy chain; accountability for total cost of care rather than encounter volume.
Technology	Is it possible?	The data substrate. Makes population health management, VBC contract execution, equity measurement, and strategic planning analytically feasible. Without data infrastructure, the other pillars are managing blind.	Attribution lists, TCOC measurement, HEDIS stratification, risk stratification, care gap identification, AI-assisted clinical workflows — all the analytical functions that VBC and population health management require.
Operations	Is it executable?	The execution layer. Translates statutory mandates, payment models, data platforms, and clinical programs into organizational reality. The most analytically sound framework fails if the implementing organization lacks capacity, infrastructure, and management discipline.	Hospital transformation plans that are implemented, not filed; AHS organizational capacity sufficient to manage 14 simultaneous transformation processes; project management infrastructure that tracks progress against statutory deadlines.
Clinical	Is it effective?	The mechanism of change. Payment reform changes incentives; clinical redesign changes behavior. The financial case for preventive investment only materializes when clinical programs actually deliver the utilization reduction that produces savings.	Primary care transformation that prevents hospitalizations; behavioral health integration that reduces crisis presentations; care management that closes gaps before they become acute episodes.
Equity	Is it just?	The cross-cutting constraint and accountability lens. Not a separate program — a dimension applied to every decision in every other pillar. Transformation that improves averages while widening disparities has failed by the framework's own standards.	Stratified measurement that reveals disparities hidden in aggregates; payment designs that do not penalize providers serving high-SDOH populations; clinical programs that reach disadvantaged populations; technology infrastructure that produces disaggregated data.

Figure 1.2 — The six pillars: diagnostic questions and structural roles in the six-pillar framework. Source: six-pillar framework documentation; Acts 167 and 68 of 2025\.

The Architecture of Interdependency: The Fifteen Dependency Relationships

To

From	Policy	Technology	Economics	Clinical	Equity	Operations
Policy			Enables mandatory authority.		Enables accountability mandates.	Requires statutory deadlines.
Technology			Enables VBC financial management.	Enables risk stratification.	Enables demographic stratification.	
Economics		Requires data infrastructure.		Drives financial rationality.	Requires social risk adjustment.	
Clinical					Enables access expansion.	Requires execution infrastructure.
Equity	Constrains with justice reviews.			Constrains with cultural competency.		
Operations	Enables data feedback loops.	Requires workforce to run infrastructure.				

Figure 1.3 — The Architecture of Interdependency: The Fifteen Dependency Relationships

The six pillars do not operate independently. They interact through a set of dependencies and feedback loops that determine whether transformation produces the outcomes it promises. Understanding these interactions is the core analytical skill this book develops.

The foundational dependency is between Policy and Economics. Payment reform (Economics) requires policy authority — either statutory mandate or regulatory power — to be effective at scale. Vermont learned this lesson through a decade of voluntary payment reform efforts: the Vermont All-Payer ACO Model, OneCare's voluntary hospital participation, VITL's voluntary HIE connectivity. Each produced partial results because voluntary participation allowed the highest-cost actors to opt out. Act 68's mandatory RBP and global budget requirements are a Policy pillar response to the failure of Economics pillar interventions without Policy pillar backing.

The second critical dependency is between Economics and Clinical. Payment reform changes the financial incentive structure; clinical redesign changes the care delivery model to respond to the new incentives. A hospital operating under a global budget has a financial incentive to prevent hospitalizations — but it can only act on that incentive if it has the primary care network, care coordination infrastructure, and behavioral health integration to actually keep patients out of the hospital. Without the Clinical pillar investment, the Economics pillar incentive produces financial pressure without clinical improvement.

The third dependency is between Technology and all other pillars. You cannot attribute patients to hospitals for global budget purposes without accurate patient matching across providers. You cannot identify care gaps for quality improvement without population-level claims and clinical data. You cannot model the financial impact of service line closures without analytics infrastructure. You cannot monitor equity metrics without data stratified by race, income, and geography. Technology is the information substrate on which every other pillar's management functions depend.

The Equity pillar has a different kind of dependency: it is simultaneously a constraint on every other pillar and a beneficiary of all of them. Every payment reform intervention, clinical redesign, and technology investment must be evaluated for its equity implications — does it reduce disparities or exacerbate them? Does it reach the populations that need it most or concentrate benefits among those already well-served? Vermont's Act 168 goals explicitly include reducing health inequities alongside reducing costs and improving outcomes, recognizing that transformation that achieves the first two without the third is incomplete.

The Operations pillar is the translation layer — the organizational capacity that converts policy intent, payment incentives, technology tools, clinical models, and equity goals into implemented programs with trained staff and measured outcomes. A transformation strategy that is analytically sound, financially compelling, technically feasible, and clinically validated will still fail if the organization responsible for executing it lacks the project management infrastructure, the change management capability, and the sustained leadership attention that execution requires.

These five relationships are the framework's structural core. What follows develops the complete architecture: all fifteen directed dependency relationships between and among the six pillars, the specific mechanisms by which each dependency operates, the failure modes that emerge when dependencies are ignored, and the Vermont evidence that confirms the theoretical structure with empirical specificity. The six-pillar framework map — available as an interactive visualization on the technical infrastructure framework — renders these fifteen relationships as a navigable diagram.

Policy Dependencies: Three Relationships That Establish the Mandatory Architecture

Policy is the pillar from which mandatory authority flows. Its three outbound dependencies — to Economics, Operations, and Equity — are among the most critical in the framework, because they are the relationships that convert every other pillar from optional to required.

Policy → Economics: Mandatory authority enables structural payment reform

Dependency type: ENABLES (critical path)

The most important dependency in the entire framework. Payment reform that is voluntary produces voluntary results. Vermont spent a decade under OneCare Vermont — a voluntary ACO model — and produced modest results among participating hospitals while non-participating hospitals continued operating under fee-for-service without consequence. Act 68 of 2025 changed the architecture: reference-based pricing is mandatory for all Vermont hospitals beginning FY2027, and global budgets are mandatory beginning FY2028. This is not an incremental improvement on the voluntary model. It is a structural change that eliminates the opt-out that made the voluntary model ineffective.

Vermont evidence: Oliver Wyman's Act 167 diagnosis explicitly found that voluntary reform was insufficient given the financial trajectory. The $700 million to $2.4 billion five-year deficit projection assumes the continuation of the commercial pricing structure that mandatory RBP is designed to dismantle. Without the Policy pillar's mandatory authority, the Economics pillar remains aspirational.

Policy → Operations: Statutory deadlines force execution capacity

Dependency type: REQUIRES (critical path)

One of the most underappreciated dependencies in healthcare transformation. Organizations develop the capacity to execute complex initiatives when they have non-negotiable deadlines, not when they have good intentions. Act 68's December 2028 Statewide Strategic Plan deadline, the FY2027 RBP mandate, and the FY2028 global budget mandate are not just policy milestones — they are organizational forcing functions that require AHS to build the project management infrastructure, analytics capability, and staffing depth that the transformation requires.

Without the Policy pillar imposing external accountability, the Operations pillar faces the chronic problem of healthcare administration: transformation planning that is perpetually in progress but never complete, capacity building that is always on the agenda but never funded, and organizational development that yields to operational demands every time there is a conflict. Vermont's statutory deadlines are the mechanism that prevents this. They are deliberately irreversible.

Policy → Equity: Statutory mandates embed equity accountability

Dependency type: ENABLES

Acts 167 and 68 both contain explicit equity requirements — they are not aspirational statements about desired outcomes but operational mandates: equity metrics must be tracked, disparities must be reported, and the Statewide Strategic Plan must address geographic and demographic equity gaps. This is the Policy pillar enabling the Equity pillar's accountability function. Without statutory embedding, equity monitoring is a discretionary activity that competes with operational priorities. With it, equity measurement is a compliance requirement.

Vermont evidence: the Health Care Delivery Advisory Committee established under Act 68 is required to consult with the Health Equity Advisory Commission and to incorporate equity considerations into the Statewide Strategic Plan. This institutional design is the Policy pillar building the Equity pillar's accountability structure into the statutory framework.

Economics Dependencies: Three Relationships That Transmit Incentive Change

Economics is the pillar through which policy mandates are converted into behavioral change at the organizational and clinical level. Payment models do not change behavior directly — they change the financial logic that shapes decision-making. The Economics pillar's three outbound dependencies — to Clinical, Technology, and Equity — describe the mechanisms through which this conversion happens.

Economics → Clinical: Payment incentives drive care delivery redesign

Dependency type: DRIVES (critical path)

The most important economics dependency. Under fee-for-service, preventing a hospitalization costs the hospital revenue. A primary care practice that successfully manages a diabetic patient's condition and prevents a $15,000 hospitalization has generated no billable encounter for the prevented admission. The clinical logic and the financial logic point in opposite directions. Under global budgets, the calculus reverses: every avoidable hospitalization is a cost against a fixed revenue envelope, and every prevented admission is a margin improvement. The payment model makes population health management financially rational for the first time.

Vermont evidence: Vermont's Blueprint for Health demonstrates this dependency in reverse. The Blueprint's 5.8:1 return on investment — $5.8 million in reduced medical expenditure per $1 million invested in primary care — is achievable because the Blueprint's all-payer PMPM payments to PCMHs create a financial structure that rewards the clinical investment. Remove the Economics pillar and the Blueprint's clinical *model produces its results without capturing its financial returns. Sustain the Economics pillar through AHEAD's global budgets and the Clinical pillar's population health logic becomes fully incentive-aligned.*

Economics → Technology: VBC contracts require data infrastructure

Dependency type: REQUIRES

Value-based care contracts are agreements to share financial accountability for a population's total cost of care. They cannot be executed without the data infrastructure to measure that total cost. Attribution lists, TCOC benchmarks, shared savings calculations, risk adjustment factors, quality measure reporting — all of these require the all-payer claims data, the population health analytics platform, and the FHIR-based data exchange that the Technology pillar provides. An organization that signs an APM contract without adequate data infrastructure is making a financial commitment it cannot monitor, manage, or dispute.

Vermont evidence: Vermont's AHEAD global budgets for hospitals, beginning January 2027, require GMCB to set prospective annual budgets based on historical Medicare spending adjusted for population risk. This calculation requires accurate VHCURES data, validated risk adjustment methodology, and an analytics platform capable of producing hospital-specific TCOC projections. The AHS-GMCB analytics vendor procurement underway in 2025-2026 is the Technology pillar investment that makes the Economics pillar's AHEAD implementation analytically executable.

Economics → Equity: VBC design must include social risk adjustment

Dependency type: REQUIRES

Value-based care contracts that do not adjust for social risk systematically penalize providers serving high-SDOH populations. A hospital in Essex County, Vermont — serving a population with higher poverty rates, housing instability, food insecurity, and transportation barriers — faces higher care management costs and lower quality measure performance potential than a hospital in Burlington serving a more economically stable population. If their global budgets are set at the same percentage of Medicare rates without social risk adjustment, Essex County's hospital is being held to a standard that does not reflect its population's actual cost burden.

Vermont evidence: Vermont's AHEAD global budget design must address this explicitly. The methodology for social risk adjustment in Vermont's hospital global budgets is an active design question as of early 2026\. Getting it wrong — designing an Economics pillar instrument that ignores the Equity pillar's requirement — would produce a payment system that penalizes the hospitals serving the most vulnerable Vermonters.

Technology Dependencies: Three Relationships That Enable Analytical Management

Technology is the data substrate on which every other pillar's management functions depend. It has the broadest set of outbound dependencies in the framework — three enabling relationships to Economics, Equity, and Clinical that describe what becomes possible when data infrastructure is adequate, and what remains impossible when it is not.

Technology → Economics: Analytics makes VBC financial management possible

Dependency type: ENABLES

This dependency runs in both directions between Technology and Economics: the Economics pillar requires data infrastructure (documented above), and the Technology pillar enables the financial management capability that VBC contracts require. The distinction is between existence and effectiveness. An organization can enter an APM contract without adequate analytics — the contract still exists, the financial obligations are still real. But the organization cannot manage its financial performance under that contract, cannot identify where its costs are above benchmark, cannot target care management investment toward the highest-return opportunities, and cannot dispute errors in the payer's shared savings calculations.

Vermont evidence: VHCURES population health analytics make the APM financial modeling described in Chapter 7 possible. The APM Shared Savings Calculator, the Hospital Financial Stress Test Model, and the VBC Readiness Assessment — all tools in the HTR Research Lab — are built on VHCURES data logic. Vermont hospitals that invest in VHCURES analytics capability before AHEAD launches in January 2027 will be able to manage their global budgets proactively rather than reactively.

Technology → Equity: Disaggregated data reveals disparities hidden in averages

Dependency type: ENABLES (critical)

The Equity pillar's entire analytical function depends on the Technology pillar's ability to produce demographic stratification. You cannot measure a disparity you cannot see. Vermont's 91% primary care access rate — four points above the national benchmark — is a genuine achievement. It also hides an 11-point gap between white Vermont adults and BIPOC Vermont adults. That gap is invisible in aggregate data. It becomes visible only when VHCURES data is stratified by race/ethnicity, and that stratification is only possible when the data contains adequate demographic information.

Vermont evidence: Vermont's race/ethnicity data in VHCURES is incomplete — commercial claims frequently lack demographic fields. This is not a minor data quality issue; it is a Technology pillar gap that directly limits the Equity pillar's analytical capability. The Vermont Department of Health Health Equity Data Report (January 2025\) documents this limitation explicitly. Closing the Technology gap is prerequisite to the Equity pillar's disparity measurement function.

Technology → Clinical: Data infrastructure enables population health management

Dependency type: ENABLES

The clinical programs described in Chapter 10 — Blueprint PCMH, CoCM, CCBHC, CHT deployment — can all be designed and implemented without data infrastructure. What they cannot do without data infrastructure is function as population health programs rather than individual encounter programs. The difference is analytically fundamental. Risk stratification identifies which patients are most likely to deteriorate; without it, care management deploys reactively, after hospitalization rather than before. Care gap identification triggers outreach for overdue preventive services; without it, gaps close only when patients happen to schedule appointments. SDOH screening tools flag patients for CHT navigation; without the data platform to connect the flag to the navigation workflow, the flag is generated and ignored.

Vermont evidence: Blueprint's clinical registry is the Technology pillar product that enables Blueprint's Clinical pillar outcomes. The 5.8:1 ROI documented for Blueprint is achievable precisely because Blueprint combines the Clinical pillar's care team model with the Technology pillar's population health analytics. The AI scribe grants in the RHT Program are the Technology pillar's contribution to the Clinical pillar's workforce capacity problem.

Clinical Dependencies: Two Relationships That Connect Care Models to System Requirements

Clinical is the pillar that converts payment incentives into actual health outcomes. Its two outbound dependencies — to Operations and Equity — describe the operational requirements of clinical programs and their equity implications.

Clinical → Operations: Care models require execution infrastructure

Dependency type: REQUIRES

The most consistently underestimated dependency in healthcare transformation. A care model is a diagram until it has workforce, credentialing, administrative infrastructure, and operational management. The Collaborative Care Model requires a behavioral health care manager employed by a primary care practice. The CCBHC model requires a certified entity with the organizational capacity to serve all patients regardless of insurance status. Blueprint PCMHs require care coordinators, health information exchange connectivity, and NCQA recognition processes. None of these are available without Operations pillar investment.

Vermont evidence: Oliver Wyman's Act 167 analysis found that Vermont's administrative cost per adjusted discharge is 91% above the national benchmark. This is not merely an inefficiency problem — it is a signal that Vermont's Operations pillar infrastructure is absorbing cost without producing the execution capability that clinical transformation requires. The RHRC engagement with all 14 Vermont hospitals is specifically designed to build the Operations pillar capacity that Clinical pillar programs need.

Clinical → Equity: Care delivery expansion is the primary access gap mechanism

Dependency type: ENABLES

Geographic and demographic disparities in healthcare access are primarily clinical access problems — not enough primary care providers, not enough behavioral health capacity, not enough community health workers in underserved communities. The Equity pillar's access gap goals cannot be achieved without the Clinical pillar's care model expansion. Blueprint's PCMH expansion into Northeast Kingdom communities, the CCBHC network's geographic reach, and the CHT deployment in high-SDOH communities are Clinical pillar investments that directly serve Equity pillar objectives.

Vermont evidence: Essex County's 8% uninsurance rate and limited primary care access are equity problems with a clinical solution — expanding Blueprint-participating practices in underserved HSAs, deploying telehealth through the RHT Program to reduce geographic access barriers, and certifying additional CCBHCs in Northeast Kingdom communities. Remove the Clinical pillar investment and the Equity pillar's access gap remains unmeasured but unaddressed.

Operations Dependencies: Two Feedback Relationships That Close the Loop

Operations is the pillar that executes all other pillars' intentions. Its two outbound dependencies run back to Policy and Technology — feedback relationships that make the system self-correcting rather than purely top-down.

Operations → Policy: Implementation data feeds back into policy design

Dependency type: ENABLES

This is the feedback loop that allows the policy architecture to be self-correcting. AHS's monthly transformation reports to the Vermont Legislature — required under Act 68 — document whether transformation is proceeding on the statutory timeline, where gaps have emerged, and what adjustments are required. The GMCB's RBP methodology for FY2027 is being refined based on hospital financial data from the operations layer. The December 2028 Statewide Strategic Plan will be drafted based on what the Operations pillar has learned from implementing the preceding three years of transformation. This is not passive reporting — it is active policy learning.

Vermont evidence: AHS's November 2025 transformation report changed the policy understanding of Vermont's administrative cost problem by documenting the UVMMC $3,826 per adjusted discharge figure against the $1,427 benchmark. That Operations pillar data point is now embedded in the policy case for mandatory transformation. Without the operations feedback loop, policy would operate on assumptions rather than evidence.

Operations → Technology: Workforce runs the data infrastructure

Dependency type: REQUIRES

The Technology pillar's infrastructure — VHCURES analytics, the CIN data platform, FHIR compliance, AI governance programs — does not operate itself. It requires IT staff to maintain systems, data analysts to generate the reports that clinical and financial management teams use, privacy officers to manage HIPAA compliance, and cybersecurity staff to protect the infrastructure from the ransomware threats that have disabled rural health systems nationally. The Operations pillar's workforce is the human substrate on which the Technology pillar's data infrastructure runs.

Vermont evidence: Vermont's rural CAHs — Grace Cottage, Gifford Medical Center, North Country Hospital — have limited IT staff. This Operations pillar constraint directly limits their Technology pillar capability. The Vermont CIN's shared services model addresses this: a shared analytics and IT function within the CIN provides all 14 hospitals access to Technology pillar capability they cannot individually afford. The CIN is an Operations pillar solution to a Technology pillar gap.

Equity Constraints: Two Relationships That Apply the Justice Lens to the Whole System

Equity's two outbound dependencies are constraining rather than enabling — they impose requirements on Policy and Clinical that must be designed in from the beginning rather than retrofitted. Equity is not the last pillar in the sequence; it is the lens applied to every other pillar's decisions throughout the design and implementation process.

Equity → Policy: Equity accountability constrains every policy decision

Dependency type: CONSTRAINS

Every policy decision in Vermont's transformation — the RBP rate methodology, the global budget design, the COE designation framework, the Statewide Strategic Plan structure — must be evaluated for equity impact before it is finalized. This is not post-hoc equity review; it is an equity constraint embedded in the policy design process. A RBP methodology that sets rates uniformly without adjusting for the market conditions faced by hospitals in underserved communities is technically correct and practically inequitable. An equity constraint would require that rate methodology to account for geographic access implications before adoption.

Vermont evidence: Act 68's requirement that the Health Care Delivery Advisory Committee consult with the Health Equity Advisory Commission is the statutory mechanism for this constraint. The interaction is not advisory — it is structural. Equity considerations are required inputs into the policy process, not optional enhancements.

Equity → Clinical: Equity constrains how care is delivered, not just to whom

Dependency type: CONSTRAINS

Implicit bias in clinical settings, cultural incompetence, language access barriers, and differential treatment quality by race/ethnicity are clinical practice problems with equity implications. They are not solved by expanding access — expanding access to biased care widens some disparities while narrowing others. The Equity pillar constrains the Clinical pillar's design by requiring that care models address cultural competence, language access, and practice variation as design requirements, not implementation afterthoughts.

Vermont evidence: Vermont's BIPOC adults have a 79-81% primary care access rate against 91% statewide — but that gap reflects not only access barriers but differential engagement rates that are partly driven by trust and cultural competence factors in clinical settings. Blueprint's HRSN screening program is a Clinical pillar program designed with the Equity pillar's requirement that screening be linguistically and culturally appropriate and that the navigation it generates connects to resources that actually serve the communities being screened.

The Failure Cascade: What Happens When a Pillar Is Missing

The most rigorous test of a dependency framework is not whether it describes how things work when all components are present — it is whether it accurately predicts what fails when a component is removed. The fifteen dependencies described above generate specific, testable failure cascade predictions. The following analysis applies this test to each pillar.

Pillar removed	Immediate failure	Second-order failure	System-level outcome	Historical example
Policy	Payment reform remains voluntary. Highest-cost actors opt out. Rate benchmarks have no enforcement mechanism.	Clinical programs continue but without the financial incentive alignment that makes population health investment rational. Technology infrastructure is built but not used for financial management because APM contracts are not mandatory.	Incremental improvement among willing participants; no system-level change; ongoing premium inflation; hospital financial crisis continues on Oliver Wyman's trajectory.	Vermont 2010-2022: OneCare Vermont voluntary ACO. Modest results. 9/14 hospitals in losses by 2023\.
Economics	Clinical programs continue under fee-for-service incentives. Hospitals remain financially rewarded for volume, not value.	Primary care investment produces outcomes but cannot capture the financial return — the 5.8:1 ROI accrues to payers, not providers. Technology infrastructure is built but used for billing optimization rather than population health.	Clinical quality improves at the margin; costs continue rising; the financial case for transformation investment cannot be made; hospital financial fragility deepens.	U.S. national experience: 20+ years of clinical quality improvement without payment reform. Quality improved significantly; cost growth continued.
Technology	VBC contracts signed but not managed. Attribution errors go undetected. TCOC measurement is unreliable. Quality measures cannot be stratified.	Economics pillar contracts produce unexpected financial losses because the organization cannot identify where costs exceed benchmark. Equity pillar cannot measure disparities it cannot see. Clinical pillar cannot identify high-risk patients before they hospitalize.	VBC contracts produce financial losses for providers who cannot manage them; organizations exit VBC; payment reform stalls; equity gaps widen invisibly.	Most ACO failures in the early CMMI model period: organizations entered models without analytics infrastructure and produced unexpected losses.
Clinical	Payment reform changes incentives but has nothing to incentivize. Global budgets create financial pressure without the clinical programs that	Operations pillar manages financial pressure through service reductions rather than care model redesign. Equity pillar's access gaps widen as clinical capacity is cut to meet budget constraints.	Hospital financial crisis accelerates under global budget pressure without the clinical programs that make global budgets	Maryland HSCRC early period: global budgets without adequate primary care investment produced hospital financial pressure without the community

Pillar removed	Immediate failure	Second-order failure	System-level outcome	Historical example
	reduce costs sustainably.		sustainable. Rural hospitals close.	health improvement that makes global budgets sustainable long-term.
Equity	Transformation proceeds but disparities widen. Average outcomes improve while geographic and demographic gaps grow.	Policy decisions that are technically optimal produce inequitable geographic distributions of access and services. Technology infrastructure is built but not used for stratified measurement. Clinical programs improve average quality without closing disparity gaps.	System transformatio n that increases overall efficiency while reducing equity — a technically successful and ethically failed transformatio n.	National VBC experience: most APM models improved average quality; most widened racial and socioeconomic disparities in quality. The efficient provider disadvantage penalized safety-net organizations.
Operations	Statutory mandates exist on paper. Clinical programs are designed. Data platforms are procured. Nothing is implemented.	Policy accountability fails because there is no organizational capacity to meet statutory deadlines. Economics pillar contracts are signed but not managed. Technology platforms are deployed but not used.	Transformatio n documents accumulate; statutory deadlines are extended; the December 2028 Strategic Plan is descriptive rather than committal; Vermont's Oliver Wyman deficit trajectory continues.	Most state health reform efforts of the 2010s: correct policy, adequate financing, sophisticated data — organizational capacity insufficient to execute at statutory speed.

Figure 1.4 — Failure cascade analysis: what breaks when each pillar is absent. Source: the framework analysis; CMMI evaluation literature; Oliver Wyman Act 167 Report; Vermont experience.

Using the Dependency Map as an Analytical Tool

The dependency map is not merely a theoretical framework — it is a practical analytical tool for three specific management functions: transformation sequencing, risk identification, and investment prioritization.

Transformation Sequencing: What Must Come First

The dependency structure generates a partial ordering of interventions. Some investments cannot produce their expected results until enabling investments are in place. Getting the sequence wrong is not merely inefficient — it produces failed interventions that generate political resistance to subsequent correct interventions.

The correct sequencing logic from the dependency map:

- **Policy before Economics:** Mandatory authority must precede payment reform deployment. Vermont's Acts 167 through 68 establish mandatory authority before RBP and global budgets take effect. This sequencing is not coincidental — it is the precondition for payment reform's effectiveness.
- **Technology before Economics (analytics function):** The analytics infrastructure for VBC contract management should be in place before full-risk contracts are signed. Vermont hospitals entering AHEAD in January 2027 should complete HCC gap analyses and VHCURES attribution modeling before the performance year begins.
- **Operations before Clinical (capacity building):** The organizational infrastructure for care model execution should be built before care models are deployed at scale. The RHRC engagement with 14 Vermont hospitals is building Operations pillar capacity before AHEAD's clinical accountability requirements take full effect.
- **Equity throughout, not at the end:** Equity is a constraint, not a final review. Designing the RBP methodology, global budget structure, COE designations, and analytics vendor requirements without equity review is cheaper and faster — and produces inequitable instruments that require expensive redesign or produce inequitable outcomes.

Risk Identification: Reading the Dependency Map for Vulnerabilities

The dependency map identifies Vermont's transformation vulnerabilities with precision. The critical path runs through the Technology pillar's analytics capability: the AHEAD global budgets require VHCURES analytics, the equity measurement requires demographic stratification, and the clinical risk stratification requires the population health platform. The AHS-GMCB analytics vendor procurement — underway in 2025-2026 but not yet complete — is the single highest-risk gap in Vermont's current transformation architecture.

A delay in analytics vendor selection and deployment does not merely delay one program. It delays the financial management capability for AHEAD, the equity measurement capability for the Statewide Strategic Plan, and the risk stratification capability for Blueprint's CCBHC expansion. Three dependencies converge on this single Technology pillar investment.

Investment Prioritization: Where Marginal Investment Has the Highest Return

Not all pillar investments have equal leverage in the dependency structure. An investment in a pillar with many inbound dependencies — one that is required by multiple other pillars — has higher leverage than an investment in a pillar that is only required by one.

The Technology pillar has the highest inbound dependency count in Vermont's current architecture: Economics requires it, Clinical requires it, and Operations requires it. The Operations pillar has the second highest: Clinical requires it and Policy requires it. This dependency structure implies that the highest-leverage investments in Vermont's current transformation phase are:

- **Analytics vendor deployment (Technology pillar — Economics \+ Equity \+ Clinical all depend on it)**
- **AHS organizational capacity building (Operations pillar — Policy accountability \+ Clinical execution both require it)**
- **HCC gap closure at AHEAD-participating hospitals (Technology \+ Economics intersection — pre-launch analytics investment with direct financial impact at AHEAD start)**

The Vermont Thread: Six Pillars in One System

Vermont's healthcare transformation is the closest thing to a controlled experiment in simultaneous six-pillar intervention that American health policy has produced. The following table maps Vermont's primary reform vehicles onto the six-pillar framework, establishing the connective tissue that runs through every subsequent chapter.

Pillar	Primary Vermont intervention	Secondary interventions	Chapter
Policy	Acts 167 (2022), 51 (2023), 68 (2025): the legislative reform cascade; AHEAD Model State Agreement (January 2025\)	Act 55 (drug pricing); Act 49 (insurer solvency); Act 62 (HIE governance); federal RHT Program award	Chapter 4: The Policy Pillar in Practice — CMMI Models, Waiver Strategy, and the Federal-State Policy Interface
Economics	Reference-based pricing (mandatory, FY2027);	Act 68 hospital spending reduction	Chapter 6: The Economics Pillar in

Pillar	Primary Vermont intervention	Secondary interventions	Chapter
	global hospital budgets (FY2028-2030); AHEAD total cost of care accountability	(2.5%, FY2026); GMCB FY27 budget guidance (-1% commercial rate); Act 55 drug cap ($135M savings)	Practice — VBC Financial Modeling, APM Readiness, and the ROI of Transformation
Technology	VHCURES all-payer claims database; Vermont CIN (RHT Program); AHS-GMCB analytics vendor; AI scribe and RPM grants	VITL/VHIE HIE; DVHA HIT Plan (Act 62); statewide EHR feasibility assessment; FHIR compliance investments	Chapters 12 and 13
Clinical	Blueprint for Health (all-payer PCMH \+ CHT); CoCM and MHI behavioral health integration; CCBHC expansion (7 entities by 2026\)	Hub and Spoke SUD model; PACE program development; COE regionalization; dementia care infrastructure	Chapter 10: The Equity Pillar — Closing Gaps, Not Just Averaging Them
Equity	Geographic equity monitoring (HSA-level); SDOH integration through AHS HSA Coordinators; rural access preservation in RBP/global budget design	HRSN universal screening (Blueprint); CCBHC access for all populations; Northeast Kingdom focus; GLP-1 access advocacy	Chapter 12: The Technology Pillar — Data Infrastructure for a Transformed Health System
Operations	14-hospital transformation planning (RHRC \+ AHS); Vermont CIN shared services; Act 68 transformation grants ($2M); PMO establishment	EMS regionalization; administrative simplification (CON, prior auth); AHS restructuring; Division of Planning and Effectiveness	Chapters 8, 11

Figure 1.5 — Vermont's six-pillar transformation map. Sources: Acts 167, 51, 55, 62, 68 of 2022-2025; AHEAD State Agreement; Vermont RHT Program Application; Oliver Wyman Act 167 Report.

This table is not a comprehensive inventory of Vermont's healthcare reform. It is a navigation map — a way of locating Vermont's specific interventions within the analytical framework that the subsequent chapters develop. A reader working through Chapter 4 (Policy pillar) will recognize Acts 167 and 68 as the Policy pillar's legislative expression. A reader in Chapter 6 (Economics pillar) will see reference-based pricing and global budgets as the Economics pillar's operative mechanisms. The framework gives every Vermont policy intervention a structural home and every analytical concept a Vermont application.

The Framework Map as a Communication Tool

Beyond its analytical function, the six-pillar interdependency map serves a communication purpose that is equally important: it gives practitioners a shared vocabulary and a shared mental model for discussing transformation across professional boundaries.

Healthcare transformation involves clinicians, economists, data scientists, policy professionals, equity advocates, and operational managers — professionals whose training gives them deep expertise in their own domain and limited fluency in the others. The economist who designs the global budget methodology may not see the clinical care management program whose financial returns the budget depends on. The clinical informaticist building the population health platform may not see the equity requirement that stratification must be demographic, not just clinical. The policy professional drafting the Statewide Strategic Plan may not see the operations capacity constraints that determine whether the plan is executable on its statutory timeline.

The framework map makes these cross-domain dependencies visible to people whose professional training did not equip them to see them. When a clinician clicks on the Clinical pillar and sees the dependency arrow to Operations, the map communicates — without requiring an economics lecture — that their care model's execution depends on organizational infrastructure decisions being made in a different part of the agency. When a policy professional sees the Equity pillar's constraining arrows to Policy and Clinical, the map communicates that equity review belongs in the design process, not the approval process.

This communication function is not secondary to the analytical function. Healthcare transformation's most persistent failure mode is not analytical error — it is organizational silo failure, in which technically correct decisions made in isolation produce collectively incorrect outcomes. The framework map is designed to make the silos visible.

> *"The map does not tell you what to do. It tells you what to pay attention to — which decisions affect which other decisions, and in which direction. That is the beginning of coordination."*

— transformation advisory methodology

Key Concepts in This Chapter

Term	Definition
Six-pillar framework	The analytical architecture organizing healthcare transformation across six interdependent domains: Policy, Economics, Operations, Clinical, Equity, and Technology. Transformation requires all six pillars to be functional; failure in any one pillar undermines the others.
Single-pillar thinking	The failure mode in which a transformation initiative addresses one pillar — typically payment reform or clinical redesign — without attending to the constraints and dependencies created by the other five pillars.
Reform cascade	A legislatively structured sequence in which each reform intervention builds on the institutional capacity and political legitimacy created by its predecessor. Vermont's Act 167 → Act 51 → Act 68 sequence is the prototype.
Structural transformation	Change that alters the fundamental incentives, relationships, and capabilities of a system — as distinguished from incremental improvement within the existing structure. This book uses 'transformation' in this precise sense throughout.
Vermont as preview	The analytical premise that Vermont's healthcare challenges — demographic aging, hospital financial fragility, commercial cross-subsidy unsustainability, voluntary reform failure — are present nationally on a 10-20 year lag, making Vermont's responses a template for the nation's future policy toolkit.
Critical path dependency	A dependency relationship in which the failure of the upstream pillar immediately disables the downstream pillar’s core function. The Policy → Economics dependency (mandatory authority

Term	Definition
	enables payment reform) and the Economics ⟶ Clinical dependency (payment incentives drive care redesign) are critical path dependencies.
Enabling dependency	A dependency relationship in which the upstream pillar's presence expands the downstream pillar's effectiveness, but the downstream pillar can function at reduced capability without it. Technology ⟶ Clinical (data infrastructure enables population health management) is an enabling dependency: Blueprint can function without full VHCURES analytics, but its population health management capability is significantly constrained.
Constraining dependency	A dependency relationship in which the downstream pillar imposes requirements on the upstream pillar's design. Equity ⟶ Policy (equity accountability constrains policy decisions) and Equity ⟶ Clinical (equity constrains care delivery design) are constraining dependencies. Unlike enabling dependencies, these run against the primary direction of authority in the framework.
Failure cascade	The sequence of second- and third-order failures produced when a single pillar is absent or underfunded. Understanding failure cascades is essential for investment prioritization — the pillar whose absence produces the longest cascade is the highest-priority investment.
Dependency leverage	The number of other pillars that depend on a given pillar, weighted by the type of dependency. High leverage pillars — those with multiple inbound critical path dependencies — are the highest-priority investments because their failure or weakness cascades to multiple other pillars simultaneously. In Vermont's current architecture, Technology has the highest dependency leverage.
Sequencing error	The error of deploying a downstream pillar investment before the upstream enabling investment is in place. Sequencing errors produce failed interventions and political resistance to correct subsequent interventions. Vermont's AHEAD global budget launch (January 2027\) before the AHS-GMCB analytics vendor is fully deployed is a potential sequencing error in the Technology-Economics dependency.
six-pillar framework map	The interactive visualization available on the technical infrastructure framework (relevant policy monitoring resources/framework) showing all fifteen dependency relationships between the six pillars. Clicking any pillar shows its inbound and outbound dependencies with relationship type, mechanism, and Vermont evidence. Source code available as a standalone React component for integration into institutional platforms.

Sources: six-pillar framework documentation; Act 167 of 2022; Act 68 of 2025; Oliver Wyman Act 167 Community Engagement: Recommendations (August 2024); Vermont AHEAD State Agreement (January 2025); CMMI model evaluation literature; Maryland HSCRC performance data; Vermont Blueprint for Health ROI study (Population Health Management); AHS Health Care System Transformation Reports (2025); Vermont Department of Health Health Equity Data Report (January 2025).

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